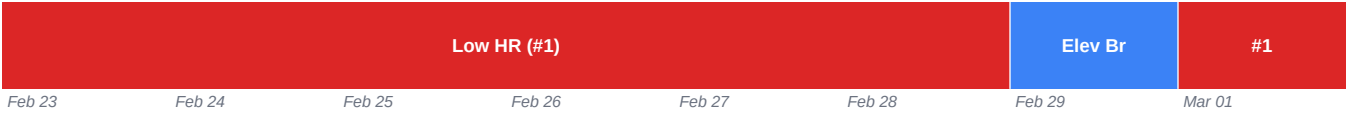


CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation
Suggested

Patient ID: PHolst Period: February 23 to March 1, 2024 Report Date: March 2, 2024 Coverage: 180/192h (85.9%)
Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 8 days from Feb 23 to Mar 01 ■ HR ■ Breathing



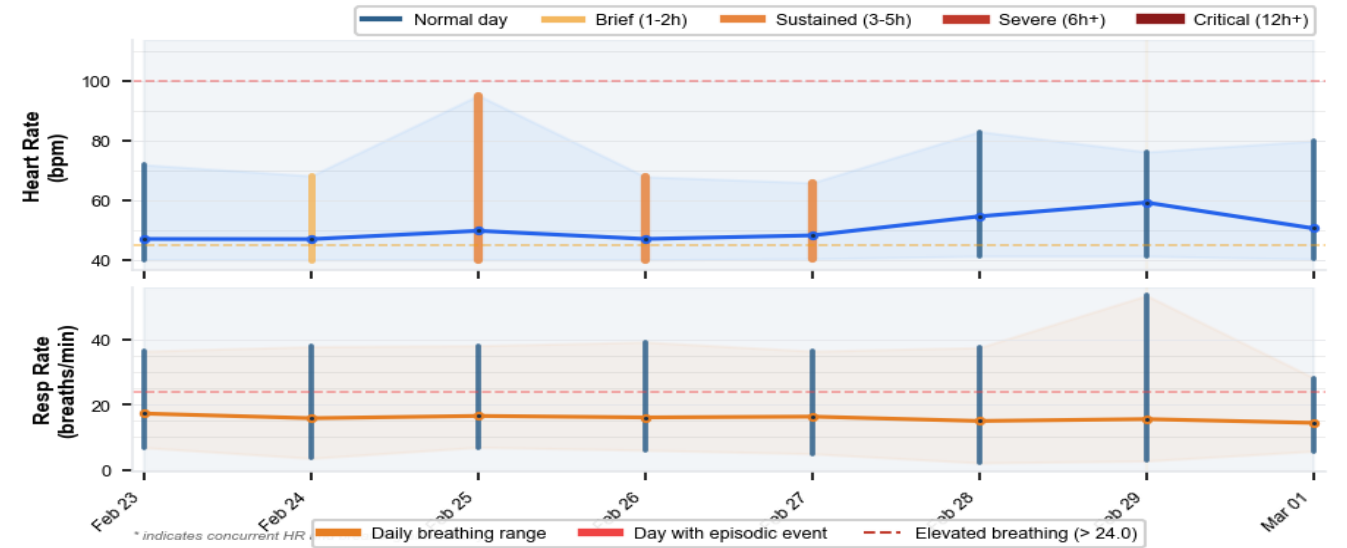
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
25	36h	Low Heart Rate	3	94%

Major Findings: Sustained low HR (avg 50, min 40)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Low Heart Rate	40 bpm	35h	3h	3	50 bpm	Feb 23 to Mar 01
2	High Breathing	33 brpm	1h	1h	1	16 brpm	Feb 29

The P5 to P95 heart rate spread was 21 bpm (43 to 64), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

- Clinical Pattern Observations:
- 1. **High variability:** HR spread 21 bpm (42 to 64).
 - 2. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.



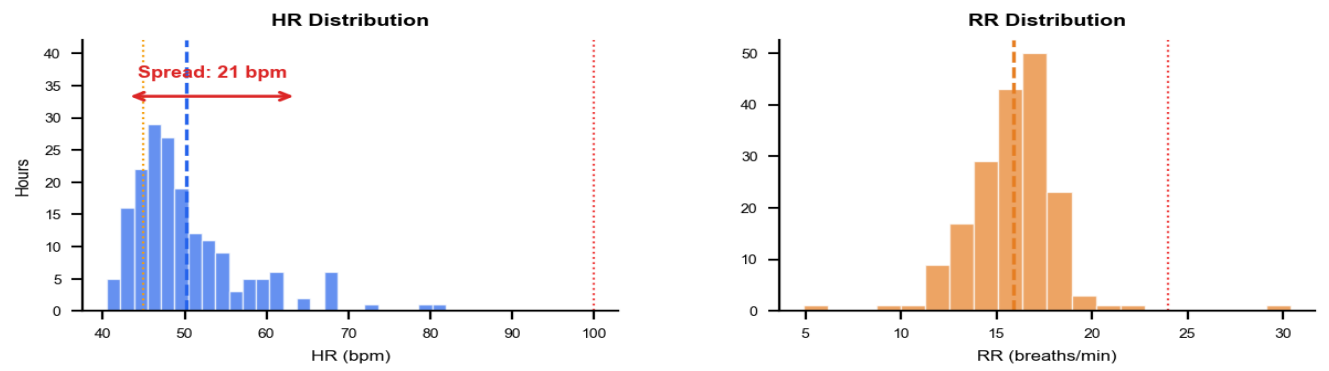
Red bars indicate days with detected episodic events.
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

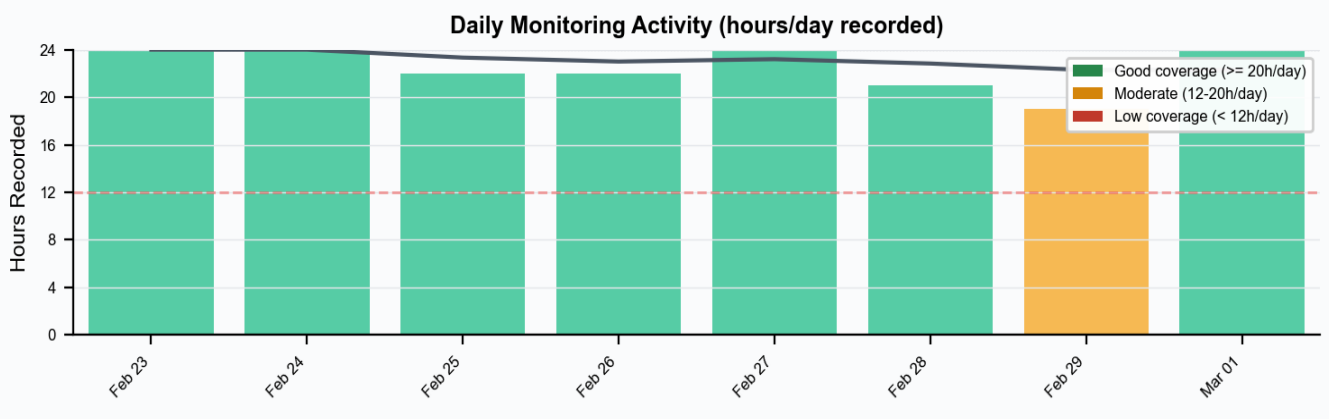
Very Low HR < 40 bpm Low HR < 45 bpm Elevated HR > 95 bpm High HR > 100 bpm
Very High HR > 110 bpm Elevated Breathing > 24 brpm High Breathing > 30 brpm Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.